

Name: ZAHMOUL, NADIA
MRN: 2018366
Encounter: 5001152770
DOB / Age: 4/27/1969 56 years
Sex / Birth Sex: Female Female
Admit Date: 11/28/2025
Discharge Date: 11/28/2025
Attending: Stoecklein,Holbrook
Copy to: CareAware Oauth,SJMCWY

Emergency Documentation

Document Type: ED Note Provider
Service Date/Time: 11/28/2025 17:59 MST
Result Status: Auth (Verified)

ZAHMOUL, NADIA

DOB: 04/27/1969
Age: 56 years
Sex: Female
MRN: 2018366
Registration Date: 11/28/2025

Basic Information

Time Seen:

Stoecklein, Holbrook 11/28/2025 16:40

Chief Complaint

Pain under ribs R Side

History Of Present Illness:

56-year-old female who reports no prior abdominal surgeries presents to the emergency department via EMS with sharp right sided flank pain. She says that this began about 5 or 6 hours ago. She says that it is very localized and nothing makes the pain feel better or worse. She denies any associated fevers, vomiting, diarrhea, hematochezia, melena, vaginal bleeding, dysuria, or hematuria. No recent trauma to the area. She called 911 and EMS administered 2.5 mg of Valium and 50 mcg of fentanyl prior to arrival. She says that neither of these have helped improve the pain. She has never had pain like this in the past. She has had 5 kidney stones and she says this feels different.

Review of Systems:

10 point review-of-systems negative except for that reviewed in the HPI.

Physical Exam

Vitals & Measurements

T: 36.3 °C (Temporal Artery) **HR:** 85 (Peripheral) **HR:** 85 (Monitored) **RR:** 23

BP: 136/86 **SpO2:** 91%

HT: 157.48 cm **WT:** 48.5 kg **BMI:** 19.56 **O2 Therapy:** Room air

Vitals and nursing note reviewed.

CONSTITUTIONAL: Constantly moving around in bed with writhing movements but not in acute distress. Well-developed and well-nourished.

HEENT: Normocephalic and atraumatic. No scleral icterus. Extraocular movements normal. Pupils are equal, round, and reactive to light.

NECK: No tracheal deviation. No stridor.

CARDIOVASCULAR: Normal rate and regular rhythm. No murmur heard. No friction rub. No gallop. Radial pulses strong and symmetric.

RESPIRATORY: Respiratory effort is normal. No respiratory distress. Normal breath

Problem List/Past Medical History

Ongoing

Anemia

Chronic rhinitis

Facial pressure

Nasal septal perforation

Sjogren syndrome

Historical

Chronic sinusitis

Facial pain syndrome

Nasal septal spur

Procedure/Surgical History

- Sinus X 2

Medication Administration

Given

droperidol, IV Push

iopamidol(iopamidol 76% injectable solution), IV Contrast

ketorolac, IV Push

Allergies

No Known Allergies

Social History

Alcohol

Use:Never

Electronic Cigarette/Vaping

Electronic Cigarette Use:Never

Substance Use

Use:Never

Tobacco

Smoking tobacco use:Never tobacco user

St. John's Medical Center

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sounds.

ABDOMINAL: Abdomen is soft and non-distended. There is no mass. There is no abdominal tenderness. There is no guarding or rebound tenderness.

MUSCULOSKELETAL: No edema. Normal range of motion. No obvious deformities.

SKIN: Warm and dry with no rashes or ecchymosis.

NEUROLOGICAL: Alert and oriented to person, place, and time. Speech is normal.

PSYCHIATRIC: Extremely anxious, psychomotor agitation.

Family History

Healthy adult: Mother and Father.

Diagnostic Results

No qualifying data available.

Medical Decision Making:

56-year-old female with no prior abdominal surgeries presents to the emergency department with 5 hours of right flank pain that is localized with no associated symptoms. She is afebrile with normal vital signs on arrival. On exam she has no significant abdominal tenderness but is moving around uncontrollably in bed. She has already been administered Valium and fentanyl by EMS. She was given Toradol. Differential diagnosis includes ureterolithiasis, cholelithiasis, cholecystitis, choledocholithiasis, pancreatitis, ovarian torsion, hemorrhagic ovarian cyst. Vascular dissection, perforation, or obstruction also considered but seem less likely. Labs including CBC, CMP, and lipase are all unremarkable with the exception of lipase which is slightly elevated at 490. This is not above the clinically relevant range for pancreatitis especially given the patient's report of very localized right flank pain. Urinalysis shows no evidence of infection but does show some hematuria. The patient is feeling better after receiving droperidol as the Toradol did not initially help, but is still having some pain and would like to proceed with CT scan. The patient was signed out to Dr. Greenbaum who will follow-up on CT results but the likely disposition will be discharge unless there is some acute finding on CT.

Assessment/Plan

1. Hematuria | (R31.9)

Orders:

droperidol, 1.25 mg= 0.5 mL, IV Push, Once, (Completed)

iopamidol(iopamidol 76% injectable solution), 65 mL, IV Contrast, Once, (Completed)

ketorolac, 15 mg= 1 mL, IV Push, Once, (Completed)

CT Abdomen and Pelvis w/ Contrast, 11/28/25 17:33:00 MST, Stat, Reason for exam severe acute right flank pain, hematuria, hx of kidney stones, Transport Mode: Stretcher, 11/28/25 17:33:00 MST, (Ordered)

Patient Education

Abdominal Pain, Adult

Blood in the Pee (Hematuria) in Adults: What to Know

Follow Up

With	When	Contact Information
Return to Emergency Department	Within 1 to 2 days, only if needed	
Additional Instructions: Return to ED if symptoms worsen		

Medication Reconciliation

Unchanged

dextroamphetamine-amphetamine (dextroamphetamine-amphetamine 10 mg oral capsule, extended release)1 Capsules Oral (given by mouth) every morning.

Refills: 0.

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dextroamphetamine-amphetamine (dextroamphetamine-amphetamine 10 mg oral tablet)1 tab Oral (given by mouth) 2 times a day. Refills: 0.

estradiol (estradiol 0.025 mg/24 hours twice weekly transdermal film, extended release)Topical (on the skin) Mon/Tue/Thu/Fri/Sat.

FLUoxetine (FLUoxetine 20 mg oral capsule)1 Capsules Oral (given by mouth) every day. Refills: 0.

FLUoxetine (FLUoxetine 20 mg oral capsule)

lisdexamfetamine (Vyvanse 70 mg oral capsule)1 Capsules Oral (given by mouth) every morning.

omeprazole (omeprazole 40 mg oral delayed release capsule)1 Capsules Oral (given by mouth) every day. Refills: 0.

omeprazole (omeprazole 40 mg oral delayed release capsule)1 Capsules Oral (given by mouth) every day. Refills: 0.

progesterone (progesterone 100 mg vaginal insert)

propranolol (propranolol 40 mg oral tablet)

sucralfate (Carafate 1 g oral tablet)1 tab Oral (given by mouth) 2 times a day. Refills: 0.

Lab Results**CBC and Differential****LATEST RESULTS**

WBC	11/28/25 16:48	7.1
RBC	11/28/25 16:48	4.11 Low
Hgb	11/28/25 16:48	12.8
Hct	11/28/25 16:48	38.1
Platelets	11/28/25 16:48	408
MCV	11/28/25 16:48	92.7
MCH	11/28/25 16:48	31.1
MCHC	11/28/25 16:48	33.6
RDW-CV	11/28/25 16:48	13.1
MPV	11/28/25 16:48	10.5
Neutro Auto	11/28/25 16:48	51.3
Lymph Auto	11/28/25 16:48	32.0
Mono Auto	11/28/25 16:48	10.8
Eos, Auto	11/28/25 16:48	4.2
Basophil Auto	11/28/25 16:48	1.4
Neutro Absolute	11/28/25 16:48	3.62
Lymph Absolute	11/28/25 16:48	2.26
Mono Absolute	11/28/25 16:48	0.8
Eos Absolute	11/28/25 16:48	0.30

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Baso Absolute 11/28/25 16:48 0.10

Routine Chemistry

LATEST RESULTS

Sodium Level	11/28/25 16:48	135
Potassium Level	11/28/25 16:48	3.8
Chloride Level	11/28/25 16:48	106
CO2	11/28/25 16:48	24.0
Alk Phos	11/28/25 16:48	53
AST	11/28/25 16:48	48 High
ALT	11/28/25 16:48	15
BUN	11/28/25 16:48	16.0
Glucose Level	11/28/25 16:48	126 High
Creatinine Level	11/28/25 16:48	0.63
eGFR CKD-EPI	11/28/25 16:48	104
Calcium Level	11/28/25 16:48	9.3
Protein Total	11/28/25 16:48	6.9
Albumin Level	11/28/25 16:48	4.4
Globulin	11/28/25 16:48	2
A/G Ratio	11/28/25 16:48	2
Bilirubin Total	11/28/25 16:48	<0.1 Low
Anion Gap	11/28/25 16:48	9 Low
Lipase Level	11/28/25 16:48	490 High
Estimated Creatinine Clearance	11/28/25 17:14	76.34

UA Macroscopic

LATEST RESULTS

UA Color	11/28/25 17:18	Yellow
UA Appear	11/28/25 17:18	Clear
UA pH	11/28/25 17:18	5.5
UA Spec Grav	11/28/25 17:18	1.028
UA Glucose	11/28/25 17:18	Negative
UA Bili	11/28/25 17:18	Negative
UA Ketones	11/28/25 17:18	15 Abnormal
UA Blood	11/28/25 17:18	Small Abnormal
UA Protein	11/28/25 17:18	Trace Abnormal
UA Urobilinogen	11/28/25 17:18	0.2
UA Nitrite	11/28/25 17:18	Negative
UA Leuk Est	11/28/25 17:18	Trace
UA Micro Ind?	11/28/25 17:18	Micro Indicated
UA Culture Indicated?	11/28/25 17:18	Not Required

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UA Microscopic	LATEST RESULTS	
UA WBC	11/28/25 17:18	0-5
UA RBC	11/28/25 17:18	6-10 Abnormal
UA Bacteria	11/28/25 17:18	None Seen
UA Epi Cells	11/28/25 17:18	6-10 Abnormal
UA Casts	11/28/25 17:18	0-2

Electronically Signed on 11/28/2025 17:59 MST

Stoecklein, Holbrook